

WHITMORE, Harold James

DOB: March 8, 1945 | Age: 80 yrs | Sex: Male | MRN: SMC-04471932
Encounter Date: May 6, 2025 | Provider: Raymond Foster, MD — Cardiology | Doc ID: SMC-2025-0506-DC

Hospital Discharge Summary

Heart Failure Exacerbation with Atrial Fibrillation (RVR) — Current Status

Admission Summary

Admitted / Discharged	May 1, 2025 / May 6, 2025 (5 days)
Attending	Raymond Foster, MD (Cardiology); co-managed Nephrology
Reason for Admission	Dyspnea, weight gain, and palpitations
Discharge Disposition	Home with HF clinic and home-health nursing

History & Presentation

80-year-old man with HFrEF (EF 30%), permanent AF, CKD G4, T2DM, COPD, treated prostate cancer presented with 5 days of increasing dyspnea, orthopnea, 3.5 kg weight gain, and palpitations. Found in AF with rapid ventricular response (rate ~130) and volume overload.

Hospital Course

Rate controlled and diuresed with IV furosemide (transient increase over home dose) to euvolemia; net −4.0 kg. Telemetry confirmed AF with controlled rates after carvedilol optimization plus low-dose digoxin (renally dosed) added for rate control. Renal function fluctuated with diuresis (creatinine peak 2.4, discharge 2.2; eGFR 29). Potassium managed (spironolactone briefly held, resumed at reduced frequency). COPD stable. Anticoagulation continued. Primary-prevention ICD remains under EP discussion; given CKD/comorbidity and patient preference, decision deferred pending goals-of-care conversation. Palliative-supportive care consult engaged for symptom-focused planning alongside disease-directed therapy.

Discharge Diagnoses

- 1. Acute on chronic decompensated HFrEF (EF 30%).
- 2. Atrial fibrillation with RVR, now rate-controlled.
- 3. CKD stage G4 (eGFR 29), cardiorenal.
- 4. T2DM; COPD (GOLD 2); osteoporosis; prostate cancer in remission; CAD s/p DES; prior TIA; anemia of CKD.

Discharge Labs

Test	Result	Units	Reference Range	Flag
Creatinine	2.2	mg/dL	0.7 - 1.3	H
eGFR (CKD-EPI)	29	mL/min/1.73 m2	> 60	L
Potassium	4.7	mmol/L	3.5 - 5.1	
BNP	510	pg/mL	< 100	H
Hemoglobin	10.6	g/dL	13.5 - 17.5	L
Digoxin level	0.8	ng/mL	0.5 - 0.9 (HF target)	

Discharge Medications (Reconciled)

Medication	Dose	Route	Frequency	Indication
Furosemide	60 mg	PO	Once daily (increased)	Congestion
Sacubitril/valsartan	24/26 mg	PO	Twice daily (reduced, renal)	HFrEF

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Medication	Dose	Route	Frequency	Indication
Carvedilol	12.5 mg	PO	Twice daily	HFrEF / rate control
Spironolactone	25 mg	PO	Every other day (reduced)	HFrEF (K monitoring)
Digoxin	0.0625 mg	PO	Once daily (renal dose)	AF rate control
Empagliflozin	25 mg	PO	Once daily	HF / CKD
Apixaban	5 mg	PO	Twice daily	AF (verify renal dosing)
Metformin	DISCONTINUE D	—	—	eGFR <30 — stopped
Insulin glargine	12 units	SC	Nightly	T2DM (replaces metformin)

Follow-up & Instructions

- HF clinic in 5-7 days; nephrology in 1-2 weeks; BMP in 5 days (renal/K after med changes).
- Daily weights; sodium <2 g/day; fluid restriction 1.5 L/day. Report weight gain >2 kg in 3 days or worsening dyspnea.
- Metformin STOPPED (eGFR <30); basal insulin started — glucose monitoring and diabetes education arranged.
- Goals-of-care follow-up with palliative care and family; ICD decision pending.
- Continue all specialist co-management. This summary reflects the patient's current clinical status as of discharge.